HEALTH PROMOTION AND PRIMARY HEALTH CARE

HEALTH CARE

Health care is an expression of concern for fellow human beings. It is defined as a "multitude of services rendered to individuals, families or communities by the agent of the health service or professions, for the purpose of promoting, maintaining, monitoring or restoring health". Such services might be staffed, organized, administered and financed in every imaginable way, but they all have one thing in common: people are being "served", that is, diagnosed, helped, cured, educated and rehabilitated by health personnel. In many countries, health care is completely or largely a government function.

Health care includes "medical care". Many people mistakenly believe that both are synonymous. Medical care is a subset of a health care system. The term "medical care (which ranges from domiciliary care to resident hospital care) refers chiefly to those personnel services that are provided directly by physician or rendered as a result of the physician's instructions".

Health care has many characteristics; they include:

1. Appropriateness (relevance) i.e., whether the service needed at all in relation to essential human needs, priorities and policies;
2. Comprehensiveness i.e., whether there is an optimum mix of preventive , curative and promotional services;
3. Adequacy, i.e., if the service is proportionate to requirement;
4. Availability , i.e., ratio between the population of an administrative unit and the health facility (e.g., population per centre; doctor population ratio);
5. Accessibility, i.e., this may be geographic accessibility, economic accessibility or cultural accessibility;
6. Affordability, i.e., the cost of health care should be within the means of the individual and the state; and
7. Feasibility, i.e., operational efficiency of certain procedure, logistic support, manpower and material resources.

HEALTH SYSTEM

The "health system" is intended to deliver health services; in order words, it constitutes the management sector and involves organizational matters, e.g., planning, determining priorities, mobilizing and allocation resources, translating policies into services, evaluation and health education.

The components of the health system include: concepts (e.g., health and disease); ideas (e.g., equity, coverage, effectiveness, efficiency, impact); objects (e.g., hospitals, health centres, health programmes) and persons (e.g., providers and consumers). Together, these form a whole in which all the components interact to support or control one another. The aim of a health system is health development- a process of continuous and progressive improvement of the health status of a population. The goal of the health system had been to achieve "Health for all" by the year 2000.

LEVELS OF HEALTH CARE

Health services are usually organized at three levels, each level supported by a higher level to which the patient is referred. These levels are:

A) **Primary health care:** this is the first level of contact between the individual and the health system where "essential" health care (primary health care) is provided. A majority of prevailing health complaints and problems can be satisfactorily dealt with at this level. This level of care is closest to the people. In the Indian context, this care is provided by the primary health centres and their sub-centres, with community participation.

B) **Secondary health care:** at this level, more complex problems are dealt with. This care comprises essentially curative services and is provided by the district hospitals and community health centres. This level serves as the first referral level in the health system.

C) **Tertiary health care:** this level offers super specialist care. This care is provided by the regional/central level institutions. These institutions provide not only highly specialized care. But also planning and managerial skills and teaching for specialized staff. In addition, the tertiary level supports and complements the actions carried out at the primary level.

HEALTH TEAM CONCEPT

It is recognized that the physician of today is overworked professionally. It is also recognized that many of the functions of the physician can be performed by auxiliaries, given suitable training. An auxiliary worker has been defined as one "who has less than full professional qualifications in a particular field and is supervised by a professional worker". The WHO no longer uses the term "paramedical" for the various health professions allied with medicine.

The practice of modern medicine has become a joint effort of many groups of workers, both medical and nonmedical, viz, physicians, nurses, social workers, health assistants, trained dais, village health guides and a host of others. The composition of the team varies. The hospital team is different from the team that works in the community health work team, it is important for each team member to have a specific and recognized function in the team and to have freedom to exercise his or her particular skills. In this context, a health team has been defined as "a group of persons who share a common health goal and common objectives, determined by community needs and towards the achievement of which each member of the team contributes in accordance with her/his competence and skills, and respecting the functions of the other". The auxiliary is an essential member of the team. The team must have a leader. The leader adequately and should know the motivations of each member in order to stimulate and enhance their potentialities. The health team concepts has taken a firm root in the delivery of health services both in the developed and developing countries. The health team approach aim, produce the right "mix" of health personnel for providing full health coverage of the entire population. The Mere presence of a variety of health professionals is not sufficient to establish teamwork; it is the proper division and combination of their operations from which the benefits of divided labour will be derived.

HEALTH FOR ALL

After three decades of trial and error and dissatisfaction in meeting people e's basic health needs, the World Health Assembly in May 1977, decided that the main social goal of government and WHO in the coming years should be the "attainment by all the people of the world by the year 2000 AD of a level of health that will permit them to lead a socially and economically productive life". This goal has come be popularly known as "Health for All by the year 2000" (HFA). The background to this "new" philosophy was the growing concern about the unacceptably low levels of health status of the majority of the world's population especially the rural poor and the gross disparities in health between the rich and poor, urban and rural population, both between and within countries. The essential principle of "HFA" is the concept of "equity in health", that is, all people should have an opportunity to enjoy good health.

PRIMARY HEALTH CARE

Primary health care conveys varying ideas from varying dimensions from natural history of disease point of view primary health care is the care for primary level prevention which is preventive and promotive in nature. It helps people to understand their basic needs, take desirable action to live as healthy as possible and seek medical assistance and care immediately when required to lead useful and productive life.

DEFINITION

According to WHO and UNICEF, primary health care is essential healthcare and technology based on practical, scientifically sound and socially acceptable methods and technology, made universally accessible to individuals and families in community by mean, acceptable to them, through their full participation and at a cost that the community and country can afford.

The concept of primary health care came into lime-light in 1978 following an international conference in Alma-Ata, USSR. It has been defined as: "essential health care based on practical, scientifically sound and socially acceptable methods and technology made accessible to individuals and families in the community through their full participation and at a cost that the community and the country can afford to maintain at every stage of their development in the spirit of self- determination".

The primary health care approach is based on principles of social equity, nation-wide coverage, self-reliance, intersectoral coordination, and people's involvement in the planning and implementation of health programmes in pursuits of common health goals. This approach has been
described as "Health by the people" and "placing people's health in people's hands". Primary health care was accepted by the member countries of WHO as the key to achieving the goal of HFA by the year 2000 AD.

The declaration of Alma-Ata stated that primary health care includes at least:

i. education about prevailing health problems and methods of preventing and controlling them;
ii. promotion of food supply and proper nutrition:
iii. an adequate supply of safe water and basic sanitation;
iv. Maternal and child health care, including family planning;

v. Immunization against infectious diseases.
vi. Prevention and control of endemic diseases.
vii. Appropriate treatment of common diseases and injuries; and
viii. Provision of essential drugs.

The concept of primary health care involves a concerted effort to provide the rural population of developing countries with at least the bare minimum of health services. The list can be modified to fit local circumstances. For example, some countries have specially included mental health, physical handicaps, and the health and social care of the elderly. The health care approach integrate at community level all the factors required for improving the health status of the population. As a signatory to the Alma –Ata Declaration, the Government of India, has the implementation of primary health care in India includes shortage of health manpower, entrenchment of a curative culture within the existing health system, and a high concentration of health services and health personnel in urban areas.

HEALTH CARE SERVICES

The purpose of health care services is to improve the health status of the population. The goals to be achieved have been fixed in terms of mortality and morbidity reduction, Increase in expectation of life, decrease in population growth rate, improvements in nutritional status, provision of basic sanitation, health manpower requirements and resources development and certain other parameters such as food production, literacy rate, reduced levels of poverty, etc.

The scope of health services varies widely from country to country and influenced by general and ever changing national, state and local health problems, needs and attitudes as well as the available resources to provide these services. A comprehensive list of health services may be found in the Report of the WHO Expert Committee (1961) on "Planning of Public Health Services".

There is now broad agreement that health services should be (a) comprehensive (b) accessible (c) acceptable (d) provide scope for community participation, and (e) available at a cost the community and country can afford. These are the essential ingredients of primary health care which forms an integral part of the country's health system, of which it is the central function and main agent for delivering health care.

PRICINCIPLE OF PRIMARY HEALTH CARE

There are various principles regarding operational aspect of primary health care.

Equitable distribution

It means that primary healthcare services must be shared equally by all the people irrespective of their ability to pay, belonging to urban and rural areas and to any segment of the community but giving priority to the unprivileged areas of the society.

Coverage and Accessibility

Primary healthcare aims at providing essential healthcare to whole population. It implies providing healthcare services to all which are required by them. E.g. to children, mother, adults, elderlies and also which are reachable to them, i.e. geographically, financially, culturally and functionally.

Community participation

It is a process by which individual and families assume responsibilities for their own health and welfare. And for those of the community, develop the capacity to contribute to their and country's development. This process creates awareness among people about their health situation and resource and motive them to solve their common problems. There are many ways in which community can participate in every stage of primary healthcare. It can be involved in the assessment of health situation, defining of health problem, health needs, setting of priorities, planning of alternative action, implementation of actions, implementation of actions by the people, monitoring and evaluation and feedback.

Multisectorial approach

No sector involved in socioeconomic development can function properly in isolation. Activities of any one sector have impact on goal of any sector. There is need for consultation and coordination of the intersectoral activity. So is true with health sector. The other sectors include agriculture, animal husbandry, water supply, sanitation, public works, communication, education and mass media, panchayats.

Appropriate health technology

Appropriate health technology is very important factor for successful primary healthcare. It implies use of method, technologies and equipment which are scientifically sound but simple in accordance to local culture so that these are understood and acceptable to those who use and to those for whom, these are used.

Human resource

Human resource is very important factor for the success of primary healthcare. Often this resource is not used effectively and sufficiently. For effective implementation of PHC, it is very essential to make full use of all available resource including the human potential of the entire community. This is possible through active involvement of people, helping them to develop their competencies to deal with their problem and become full member of healthcare aspect of community as a whole. It is important to ensure availability of adequate number of appropriate health personnel in PHC.

Services by community health workers and traditional health practitioners

Primary healthcare is the first level care which is provided by community health worker, who form a link between community people and health system. They are given short and simple training to be able to take care of some of the simple and basic health needs of people.

Referral system

It is essential that is given support of higher level health persommel which has specialized technical knowledge and technology which is useful to servive the life of the client. The transportation of people to and from referral service has to be properly organized, making most of available facilities.

Logistic of supply

Logistic of supply includes planning and budgeting for supplies required, storage, distribution and control. Supplies of the right quality and quantity have to be delivered to primary healthcare facilities. At the right time to make it possible to provide services on a continuing basis. It is advisable to have a standard list of drug and equipment which can be adjusted according to local variation, such as seasonal fluctuation and the incidence and of certain disease.

Physical facilities

The physical facilities for primary healthcare need to be simple and clean, already existing facilities can be used for purpose. If these are to be specially built, the community people can be involved to contribute their own labor and materials. It should have spacious waiting area with toilet facility.

Control and evaluation

Primary healthcare, with its supporting service has to be controlled and evaluated to ensure that it is functioning in accordance with national policy and strategy and measure are taken to improve as found necessary. Community can be involved in managerial control of primary health care. A process of evaluation has to be built in to assess the relevance, progress, efficiency, effectiveness and impact of services.

ELEMENTS OF PRIMARY HEALTH CARE

In 1978, the world health organization (WHO) adopted the declaration of Alma-Ata. The declaration, named for the host city, Almaty, Kazakhstan (formerly known as Alma-Ata), outlined the organization's stance towards health care made available for all people in the world. The declaration also defined eight essential components of primary healthcare, which helped outline a means of providing healthcare globally. In Alma-Ata declaration, it was stated to include the following essential elements in primary health care;

- Education of the people concerning prevailing health problems and methods of preventing and controlling them
- Promotion of food supply and proper nutrition
- Adequate supply of safe water and basic sanitation
- Maternal and child healthcare and family planning
- Immunization against the major infectious disease
- Prevention and control of locally endemic disease
- Appropriate treatment of common disease and injuries
- Provision of essential drug.

Public education

Public education is the first, and one of the most essential, component of primary healthcare. By educating the public on the prevention and control of health problems, and encouraging participation, the world health organization works to keep diseases from spreading on a personal level.

Proper nutrition

Nutrition is another essential component of healthcare. WHO works to prevent malnutrition and starvation and to prevent many diseased and afflictions.

Clean water and sanitation

A supply of clean, safe drinking water, and basic sanitation measures regarding trash, sewage and water cleanliness can significantly improve the health of a population, regarding and even eliminating many preventable diseases.

Maternal and child health care

Ensuring comprehensive and adequate healthcare to children and to mothers, both expecting and otherwise, is another essential element of primary healthcare. By caring for those who are at the greatest risk of health problems, who helps future generations have a chance to thrive and contribute globally. Sometimes, care for these individuals involves adequate counselling on family planning and safe sex.

Immunization

By administering global immunizations, WHO works to wipe out major infectious diseases, greatly improving overall health globally.

Local disease control

Prevention and control of local diseases is critical to promoting primary health care in population. Many disease vary based on the location. Taking these diseases into account and initiating measures to prevent them are key factors in efforts to reduce infection rate.

Accessible treatment

Another important component of primary healthcare is access to appropriate medical care for the treatment of diseases and injuries. By treating diseases and injury right away, care givers can help avoid complications and the expense of later, more extensive, medical treatment.

Drug provision

By providing essential drugs to those who need them, such as antibiotics to those with infections, caregivers can help prevent diseases from escalating. This makes the community safer, as there is less chance foe diseases to be passed along.

HEALTH CARE SYSTEMS

The health care system is intended to deliver the health care services. It constitutes the management sector an**d** involves organisational matters. It operates in the context of the socioeconomic and political framework of the country. In India, it is represented by five major sectors or agencies which differ from each other by the health technology applied and by the source of funds for operation. These are:

1. **PUBLIC HEALTH SECTOR**

a) Primary Health Care
   - Primary health centres
   - Sub centres

b) Hospitals/Health Centres

- Community health centres
- Rural hospitals
- District hospital/health centre
- Teaching hospitals

c) Health Insurance Schemes
- Employees State Insurance
- Central Govt. Health Scheme

d) Other agencies
- Defence services
- Railways

2. **PRIVATE SECTOR**

a) Private hospitals, polyclinics, Nursing homes, and dispensaries

b) General practitioners and clinics

3. **INDIGENOUS SYSTEMS OF MEDICINE**
- Ayurveda and Siddha
- Unani and Tibbi
- Homoeopathy
- Unregistered practioners

4. **VOLUNTARY HEALTH AGENCIES**
5. **NATIONAL HEALTH PROGRAMMES**

PRIMARY HEALTH CARE IN INDIA

In 1977, the Government of India launched a Rural Health scheme, based on the principle of "placing people's health in people's hands". It is a three tier system of health care delivery in rural areas based on the recommendation of the Shrivastav Committee in 1975. Close on the heels of these recommendations an International conference at Alma-Ata in 1978, set the goal of an acceptable level of Health for All the people of the world by the year 2000 through primary health care approach. As a signatory to the Alma-Ata Declaration, the Government of India was committed to achieving the goal of Health for All through primary health care approach which seeks to provide universal comprehensive health care at a cost which is affordable.

Keeping in view the WHO goal of "Health for All" by 2000 AD, the Government of India evolved a National Health Policy based on primary health care approach. It was approved by Parliament in 1983. The National Health Policy laid down a plan of action for reorienting and shaping the existing rural health infrastructure with specific goals to be achieved by 1985, 1990 and 1995 within the framework of the Sixth (1980-85) and Seventh (1985-90) Five Year Plans and the new 20 point Programme. Steps were taken to implement the National Health Policy objectives towards achieving Health for All by the year 2000. During the last decade further development of rural health infrastructure took place in view to implement National Health Policy 2002, National Population Policy 200 and more recently National Rural Health Mission with formulation of Indian Public Health Standards.

1. **Village level**

One of the basic tenets of primary health care is universal coverage and equitable distribution of health resources. That is, health care must penetrate into the farthest reaches of rural areas, and that everyone should have access to it. To implement this policy at the village level, the following schemes are in operation:

a. ASHA Scheme
b. ICDS Scheme; and
c. Training of Local Dais

ASHA

ASHA must be resident of the village – a women (married/widow/divorced) preferably in the age group of 25 to 45 years with formal education up to eight class, having communication skill and leadership qualities. Adequate representation from the disadvantaged population group will ensure to serve such groups better. The general norm of selection is one ASHA for 100 population. In tribal, hilly and desert areas the norm could be relaxed to one ASHA per habitation.

Role and responsibilities of ASHA

ASHA will be a health activist in the community who will create awareness on health. Her responsibilities will be as follows:

1. ASHA will take steps to create awareness and provide information to the community on determinants of health such as nutrition, basic sanitation and hygienic practices, healthy living and working conditions, information on existing health services, and the need for timely utilization of health and family welfare services.
2. She will counsel women on birth preparedness importance of safe delivery, breast feeding and complementary feeding, immunization, contraception and prevention of common infections including reproductive tract infection/sexually transmitted infection and care of the young child.
3. ASHA will mobilize the community and facilitate them in accessing health and health related services available at the anganwadi/sub centre/primary health centres such as immunization, antenatal check-up, postnatal check-up, supplementary nutrition, and other services being provided by the government.
4. She will work with the village health and sanitation committee of the gram panchayat to develop a comprehensive village health plan.
5. She will arrange escort/accompany pregnant women and children requiring treatment/ admission to the nearest pre-identified health facility i.e., primary health centre/.community health centre/First Referral Unit.
6. ASHA will provide primary-medical care for minor ailments such as diarrhoea, fevers, and first-aid for minor injuries. She will be a provider of directly served treatment short-course (DOTS) under revised national tuberculosis control programme
7. She will also act as a depot holder for essential provisions being made available to every habitation like oral rehydration therapy, iron folic acid tablet, chloroquine, disposable delivery kits, oral pills and condoms etc. A drug kit will be provided to each ASHA. Contents of the kit will be based on the recommendations of the expert/technical advisory group set up by the government of India, and include both AYUSH and allopathic formulations.

8. Her role as a provider can be enhanced subsequently. States can explore the possibility of graded training to her for providing new born care and management of a range of common ailments, particularly childhood illness.
9. She will inform about the births and deaths in her village and any unusual health problems/disease out breaks in the community to the sub-centre/primary health centre.
10. She will promote construction of household toilets under total sanitation campaign.

Role and integration with Anganwadi

Anganwadi worker will guide ASHA in performing following activities:

(a) Organizing Health Day once/twice a month. On health day, the women, adolescence girls and children from the village will be mobilized for orientation on health related issues such as importance of nutritious food, personal hygiene, care during pregnancy, importance of antenatal check-up and institutional delivery, home remedies for minor aliment and importance of immunization etc. AWWs will inform ANM to participate and guide organizing Health Days at anganwadi centre;

(b) AWWs and ANMs will act as resource persons for the training of ASHA;

(c) IEC activity through display of posters, folk dances etc, on these days can be undertaken to sensitize beneficiaries on health related issues;

(d) Anganwadi will be depot holder for drug kits and will be issuing it to ASHA. The replacement of the consumed drugs can also be done through AWW;

(e) AWW will update the list of eligible, couples and also the children less than one year of age in the village with the help of ASHA; and

(f) ASHA will support the AWW in mobilizing pregnant and lactating women and infants for nutrition supplement. She would also initiative for bringing the beneficiaries from the village on specific days of immunization, health check-ups/health days etc. to anganwadi centres.

Role and integration with ANM

Auxiliary Nurse Midwife (ANM) will guide ASHA in performing following activities:

(a) She will hold weekly/fortnightly meeting with ASHA and discuss the activities undertaken during the week/fortnight. She will guide her in case ASHA had encountered any problem during the performance of her activity;

(b) AWWs and ANMs will act a resource persons for the training of ASHA;

(c) ANMs will inform ASHA regarding date and time of the outreach session and will also guide her for bringing the beneficiary to the outreach session;

(d) ANM will participate and guide in organizing the Health Days at anganwadi centre;

(e) She will take help of ASHA in updating eligible couple register of the village concerned;
(f) She will utilize ASHA in motivating the pregnant women for coming to sub-centre for initial check-ups. She will also help ANMs in bringing married couples to sub-centres for adopting family planning;

(g) ANM will guide ASHA in motivating pregnant women for taking full course iron and folic acid tablets and tetanus toxoid injections etc.;

(h) ANMs will orient ASHA on the dose schedule and side effects of oral pills;

(i) ANMs will educate ASHA on danger signs of pregnancy and labour so that she can timely identify and help beneficiary in getting further treatment; and

(j) ANMs will inform ASHA on date, time and place for initial and periodic training schedule. She will also ensure that during the training ASHA gets the compensation for performance and also TA/DA for attending the training.

Anganwadi worker

Angan literally means a courtyard. Under the ICDS (Integrated Child Development Services) Scheme, there is an anganwadi worker for a population of 400-800. There are about 100 such workers in each ICDS Project. As of date over 7,067 ICDS blocks are functioning in the country. The anganwadi worker is selected from the community she is expected to serve. She undergoes training in various aspects of health, nutrition, and child development for 4 months. She is a part time worker and is paid an honorarium of Rs 1500 per month for the services rendered, which include health check-up including maintenance of growth chart, non-formal pre-school education and referral services. The beneficiaries are especially nursing mothers, pregnant women, other women (15-45 years), children below the age of 6 years and adolescent girls. Along with Village Health Guides, the anganwadi workers are the community's primary link with the health services and all other services for young children.

Local dais

A scheme for training of Dais was initiated during 2001-02. The scheme was implemented in 156 districts in 18 states/UTs of the country. The districts selected were on the basis of the safe delivery rate being less than 30 per cent. The scheme was extended to all the districts of EAG states. The aim was to train at least one Dai in every village with the objective of making deliveries safe.

2. **Sub-centre level**

The sub-centre is the peripheral outpost of the existing health delivery system in rural areas. They are being established on the basis of one sub-centre for every 5000 population in general and one for every 3000 population in hilly, tribal and backward areas. As of March 2015, 153,655 sub centres were established in the country.

A sub centre provides interface with the community grass root level, providing all the primary health care services. One LHV and one health assistant (male) located at PHC are entrusted with the task of supervision of six sub-centres.

INDIAN PUBLIC HEALTH STANDARDD FOR SUB-CENTRES

In order to provide quality care in these sub centres, Indian public health standards (IPHS) are being prescribed to provide basic promotive, preventive and few curative primary health care services to the community and achieve and maintain an acceptable standard of quality of care. These standard would help monitor and improve functioning of the sub centres. Currently, the IPHS for sub centre revised in 2012 is being followed. The services have been classified on

Essential (minimum assures services) or Desirable (that all states/UTs should aspire to achieve.)

Categorization of sub centres

In view of the current highly variable situation of sub centres in different parts of the country and even within the same state, they have been categorized into two types- Type A and Type B. categorization has taken into consideration various factors namely catchment area, health seeking behaviours, case load, location of other facilities like PHC/CHC/FRU/Hospitals in the vicinity of the sub centre. States shall be required to categorize their sub centre into two types as per the guidelines given below ad provide services and infrastructure accordingly. This shall result in optimum use of available resources.

Type A

Type-A sub centre will provide all recommended services except that the facilities for conducting delivery will not be available here. However, the ANMs have been trained in midwifery, they may conduct normal delivery in case of need. Of the requirement for this goes up, the sub centre may be considered for up gradation to Type B. the sub centres in the following situations may be included in this category.

1. Sub centres not having adequate space and physical infrastructure for conducting deliveries, due to which providing labour room facilities and equipment at these sub centres is not possible. However, there may still be demand for delivery services from the community in these areas e.g., sub centre located in remote, difficult, hilly, desert or tribal area, In such areas, the transport facility is likely to be poor and the population is still dependant on these sub-centres for availing delivery facilities. In such situations, ANMs would be required to conduct deliveries at homes and ANMs of these sub centres should mandatorily be Skilled Birth Attendance (SBA) trained, Such sub-centres should be identified for infrastructure up-gradation for conversion to Type 1 sub centres on priority,
2. Sub-centres situated in the vicinity of other higher health facilities like PHC/CHC/FRU/Hospital, where delivery facilities are available.
3. Sub-centres in headquarter area,

Guidelines

- The facilities for conducting delivery will not be available at these sub-centres and patients may usually be referred to nearby centres providing delivery facilities. These sub-centres should provide all other recommended services and focus on outreach services, prevalent diseases, tuberculosis, leprosy, non-communicable diseases, nutrition, water, sanitation and epidemics. It is also to be ensured that the Staff of these sub-centres is provided training in all new programmes on priority basis and refresher training is provided regularly.
- Extra payment should be provided to staff posted in difficult areas.
- If there is shortage, health worker male should be posted on priority basis in areas endemic for vector borne diseases.

4. Sub-centres where at present no delivery or occasional delivery may be taking place i.e. very low case load of deliveries. If the case load increases, these sub-centres should be considered for up gradation to Type B.

Type B (MCH sub centre) this would include following types if sub centres

a. Centrally or better located sub centres with good connectivity to catchment areas;
b. They have good physical infrastructure preferable with own building, adequate space, residential accommodation and labour room facilities;
c. They already have good case load of deliveries from the catchment areas; and
d. There are no nearby higher level delivery facilities.

Guidelines

Such sub-centres should be developed as a delivery facility and should also cater to adjacent Type A sub-centre areas for delivery purpose. Type B sub-centre, will provide all recommended services including facilities for conducting deliveries at the sub centre itself. They will be expected to conduct around 20 deliveries in a month. They should be provided with all labour room facilities and equipment including new born care corner. ANMs of these sub centre s should be SBA trained. These centres may be provided extra equipment, drugs, supplies, materials, 2 beds and budget for smooth functioning. If number of deliveries is 20 or more in a month, then additional 2 beds will be provided.

Services to be provided in a sub centre

Sub centre are expected to provide promotive, preventive, and few curative primary health care services. Keeping in view the changing epidemiological situation I the country, both types of sub centres should lay emphasis on non-communicable diseases related services.

Given the understanding of the health sub centre as mainly providing outreach facilities, whre most services are not delivered in the sub centre building itself, the site of service delivery may be at following places:

a. In the village : village health and nutrition day/immunization session
b. During house visits
c. During house to house surveys
d. During meetings and events with the community.
e. At the facility premises, it is desirable, that the sub centre should provide minimum of six hours of routine OPD services in a day for six days in a week. Wherever two ANMs are provided. It shall be ensured that one of the ANMs is available at the sub centre and the sub centre remains open for providing OPD services on all working days. Only one of them may provide outreach services at a time.

The services provided at sub centres are as follows

1. **Maternal and child health**

MATERNAL HEALTH

1. **Antenatal care:**

Essential:

A. Early registration of all pregnancies, within first trimester (before 12th week of pregnancy). However even if a woman comes late in her pregnancy for registration, she should be registered and care given to her according to gestational age

B. Minimum 4 ANC including registration. Suggested schedule for antenatal visits; 1st visit; within 12 weeks – preferably as soon as pregnancy is suspected- foe registration, history and first antenatal check-up; 2nd visit; between 14 and 26 weeks; 3rd weeks and term;
C. Associated services like general examination such as height , weight, B.P. anaemia, abdominal examination, breast examination, folic acid supplementation (in the first trimester), iron and folic acid supplementation from 12 weeks, injection TT, treatment of anaemia etc., (as per the guidelines);
D. Recording tobacco use by all antenatal mothers
E. Minimum laboratory investigations like urine test for pregnancy confirmation, Hb estimation, urine for albumin and sugar and linkages with PHC for other required tests
F. Name based tracking of all pregnant women for assured service delivery
G. Identification of high risk pregnancy cases
H. Identification and management of danger signs during pregnancy
I. Malaria prophylaxis in malaria endemic zones for pregnant women as per the guidelines of NVBDCP
J. Appropriate and timely referral of such identified cases which are beyond her capacity of management
K. Counselling on diet, rest, tobacco cessation if the antenatal mother is a smoker or tobacco user, information about dangers of exposure t second hand smoke and minor problems during pregnancy, advice on institutional deliveries pre-birth preparedness and complication readiness, danger singes, clean and safe delivery at home if called for
L. Provide information about provisions under current schemes and programmes like Janani Suraksha Yojana
M. Identify suspected RTI/STI case, provide counselling, basic management and referral services
N. Counselling and referral for HIV/AIDS
O. Name based tracking of missed and left out ANC cases

2. Intra natal care

Essential

A. Promotion of institutional deliveries
B. Skilled attendance at home deliveries when called for
C. Appropriate and timely referral of high risk cases which are beyond her capacity of management

Essential for Type B sub centre

A. Managing labour using Partograph
B. Identification and management of danger signs during labour
C. Proficient in identification and basic first aid treatment for PPH, eclampsia, sepsis and prompt referral of such cases as per Antenatal Care and Skilled Birth Attendance at Birth or SBA Guidelines and
D. Minimum 24 hr of stay of mother and baby after delivery at sub centre. The environment at the sub centre should be clean and safe for both mother and baby.

3. Post natal care

Essential

A. Initiation of early breast feeding within one hour of birth
B. Ensure post natal home visit on 0, 3,7 and 42nd days of deliveries at home and sub centre
C. Ensure 3,7, and 45th day visit for institutional delivery
D. In case of low birth weight baby additional visits are to be made on 14, 21 and 28th days
E. During post natal visits, advice regarding care of the mother, and care of the new born, and examination of the new born for signs of sickness and congenital abnormalities as per IMNCI guidelines and appropriate referral, if needed
F. Counselling for diet and rest, hygiene, contraception, essential new born care, immunization, infant and young child feeding, STI/RTI and HIV/AIDS
G. Name based tracking of missed and left out PNC cases

CHILD HEALTH

Essential

- New born care corner in the labour room to provide essential new born care : essential if the deliveries take plav4 at the sub centre level
  Essential new born care (maintain the body temperature and prevent hypothermia (provision of warmth/ Kangaroo Mother Care), maintain airways and breathing, initiate breast feeding within one hour, infection protection, cord care, and care of the eyes , as per the guidelines
- Counselling on exclusive breast feeding for 6 month and appropriate and adequate complementary feeding from 6 month of age while continuing breast feeding.
- Assess the growth and development of infant and under five children and make timely referral
- Immunization services: full immunization of all infants and children against vaccine preventable diseases
- Vitamin A prophylaxis children as per the guidelines
  Prevention and control of childhood diseases like malnutrition, infection, ARI, diarrhoea, fever, anaemia, including IMNCI strategy.

2. **Family planning and contraception**
   A. Education, motivation and counselling to adopt appropriate family planning method
   B. Provision of contraceptives such as condoms, oral pills, emergency contraceptive and IUD insertion
   C. Follow up services to eligible couples to adopting permanent method of tubectomy or vasectomy.
3. **Counselling** and appropriate referral for safe abortion services
4. **Adolescent health care :** education, counselling and referral
5. **Assistance** to school health services
6. **Water** quality monitoring
7. **Promotion of** sanitation including use of toilet and appropriate garbage disposal
8. **Field visits** by appropriate health workers for diseases surveillance, family welfare services including STI/RTI awareness
9. **Community need** assessment

10. **Curative** services for minor ailments including fever, diarrhoea worm infestation and first aid including first aid for animal bite and snake bite appropriate and prompt referral of needed. To provide AYUSH treatment
11. **Training** of Traditional Birth Attendants and ASHA/community health volunteers
12. **Co-ordinate services** of anganwadi workers, ASHA, village health sanitation committee etc.
13. **Disease surveillance,** Integrated Diseases Surveillance Project (IDSP)
    a. Surveillance about any abnormal increases in cases of diarrhoea/dysentery, fever with rigors, fever with rash, fever with jaundice or fever with unconsciousness and early reporting to concern PHC as per IDSP guidelines
    b. Immediate reporting of any cluster/outbreak based in syndromic surveillance
    c. High level of alertness for any unusual health event, reporting and appropriate action
    d. Weekly submission of report to PHC in S Form as per IDSP guideline

CONTROL OF LOCAL ENDEMIC DISEASES

Essential

a. Assisting in detection, control and reporting of endemic disease such as malaria, Kala Azar, Japanese encephalitis, Filariasis, Dengue etc
b. Assisting in control of epidemic outbreak as per programme guidelines

14. **National health programme**
    a. Communicable disease programme
        i. National AIDS control programme

        Essential

        - Condom promotion and distribution of condom to the high risk group
        - Help and guide patients with HIV/AIDS receiving ART with focus on adherence
        - IEC activities to enhance awareness and preventive measures about STIs and HIV/AIDS.PPTCT services and HIV-TB coordination

        Desirable

        - Linkage with microscopy centre for HIV-TB coordination HIV/STI counselling, screening and referral in Type B sub centre (screening in districts where the prevalence of HIV/AIDS is high)

        ii. National Vector Borne Disease Control Programme

        Essential

        - Collection of blood slides of fever patients
        - Rapid diagnostic test (RDT) for diagnosis of Pf malaria in high Pf areas
        - Appropriate anti-malarial treatment
        - Assistance for integrated vector control activities in relation to malaria, filaria, JE, dengue, Kala -Azar

etc. as prevalent in specific areas. Prevention of breeding places of vectors through IEC and community mobilization. Where filaria is lymphoedema/elephentiasis and hydrocele and their referrals to PHC/CHC for appropriate management. The disease specific guidelines issued by NVBDCP are to followed;

- Annual mass drug administration with single dose of diethyl carbamazine (DEC) + albendazole to all endemic, identification of cases of eligible population at risk of lymphatic filariasis;
- Promotion of use of insecticidal treated nets wherever supplied; and
- Record keeping and reporting as per programme guidelines.

iii. National Leprosy Eradication Programme:

Essential:

- Health education to community regarding signs and symptoms of leprosy, its complications, curability and availability of free of cost treatment:
- Referral of suspected cases of leprosy (person with skin patch, nodule, thickened skin, impaired sensation in hands and feet with muscle weakness) and its complications to PHC: and
- Provision of subsequent doses of MDT and follow up of persons under treatment for leprosy. maintain records and monitor for regularity and completion of treatment

iv. Revised National Tubrculosis Control Programme :

Essential:

- Referral of suspected symptomatic cases to the PHC/ Microscopy centre;
- Provision of DOTS at sub- centre, proper documentation and follow-up;
- Care should be taken to ensure compliance and completion of treatment in all cases; and
- Adequate drinking water should be ensured at sub-centre for taking the drugs.

v. National Leprosy Eradication Programme:

Essential:

- Health education to community regarding signs and symptoms of leprosy, its complications, curability and availability of free of cost treatment;
- Referral of suspected cases of leprosy (person with skin patch, nodule,
  thickened skin, impaired sensation in hands and feet with muscle weakness) and its complications to PHC: and
- Provision of subsequent doses of MDT and follow up of persons under treatment for leprosy, maintain records and monitor for regularity and completion of treatment.

vi. Revised National Tubrculosis Control Programme :

Essential:

- Referral of suspected symptomatic cases to the PHC/ Microscopy centre;
- Provision of DOTS at sub-centre, proper documentation and follow-up;
- Care should be taken to ensure compliance and completion of treatment in all cases; and
- Adequate drinking water should be ensured at sub-centre for taking the drugs.

Desirable:

Sputum collection centres established in sub- centre for collection and transport of sputum samples in rural, tribal, hilly & difficult areas of the country where designated microscopy centres are not available as per the RNTCP guidelines.

b. Non-Communicable Disease (NCD) Programmes

These type of services are to be provided at both types of sub-centres.

❖ National Programme for Control of Blindness (NPCB):

Essential:

(1) Detection of cases of impaired vision in house to house surveys and their appropriate referral. The cases with decreased vision will be noted in the blindness; and

(2) Spreading awareness regarding eye problems, early detection of decreased vision, available treatment and health care facilities for referral of such cases. IEC is the major activity to help identify cases of blindness and refer suspected cataract cases.

Desirable:

(1) The cataract cases brought to the district hospital by MPW/ANM/ and ASHAS; and

(2) Assisting for screening of school children for diminished vision and referral.

❖ National programme for prevention and control of deafness

Essential

(1) Detection of cases of hearing impairment and deafness during house to house survey and their appropriate referral; and

(2) Awareness regarding ear problems, early detection of deafness available treatment and health care facilities for referral of such cases

❖ National Mental Health Programme

Essential:

(1) Identification and referral of common community, and

(2) IEC activities for prevention and early mental illnesses for treatment and follow them up in detection of mental disorders and greater

participation/role of community for primary prevention of mental disorders.

- National Programme for Prevention and Control of Cancer, Diabetes, Cardiovascular Diseases and Stroke

  Essential:

  IEC activities to promote healthy lifestyle, sensitize the community about prevention of cancers diabetes, CVD and strokes, early detection through awareness regarding warning signs and appropriate and prompt referral of suspect cases
- National iodine Deficiency Disorders Control Programme:

  IEC activities to promote consumption of iodized salt by the community. Testing of salt for presence of iodine through salt testing kits by ASHAs (essential)
- In fluorosis affected (endemic) areas

  Essential:

  Identify the persons at risk of fluorosis. Suffering from fluorosis and those having deformities due to fluorosis, and referral

  Desirable:

  (3) Line listing of reconstructive surgery cases, rehabilitative intervention activities and referral services: and

  (4) Focused behaviour change communication activities to prevent fluorasis
- National Tobacco Control Programme

  Essential

  (1) Spread awareness and health education regarding ill effects of tobacco use especially in pregnant females, and non-communicable diseases where tobacco is a risk factor e g cardiovascular disease, cancers, chronic lung disease, and

  (2) Display of mandatory signage of "No Smoking" in the sub-centre.

  Desirable

  (1) Counselling for quitting tobacco:

  (2) Awareness to public that smoking is banned in public places and sale of tobacco products is banned to minors (less than 18 years) as well as within 100 yards of school and educational institutions, and

  (3) Spread awareness regarding law on smoke free public places.
- Oral health

  Desirable:

  (1) Health education on oral health and hygiene especially to antenatal and lactating mothers, school and adolescent children; and

  (2) Providing first aid and referral services for cases with oral health problems.
- Disability prevention

  Desirable

  (1) Health education on prevention of disability; and

(2) Identification of disabled persons during annual house to house survey and their appropriate referral.

- National Programme for Health Care of Elderly: Desirable:
  (1) Counselling of elderly persons and their family members on healthy ageing, and
  (2) Referral of sick old persons to PHC.

15. Promotion of medicinal herbs Desirable:

Locally available medicinal herbs/plants should be grown around the sub-centre as per the guidelines of department of AYUSH

16. **Record of vital events**

Essential

Recording and reporting of vital events including births and deaths, particularly of mothers and infants to the health authorities

17. **Coordination and monitoring:**

Coordinated services with AWWs, ASHAs, Village Health sanitation and Nutrition Committee PRI etc

18. Out reach/ Field services

a. Village Health and Nutrition Day (VHND): VHND should be organized at least once in a month in each village with the help of Medical Officer, Health Assistant Female (LHV) of PHC, HWM, HWF ASHA, AWW and their supervisory staff, PRI, self-help group etc.

b. House-to-house surveys: These surveys would be done once annually, preferably in April

c. Home visits : (1) For skilled attendance at birth - where the woman has opted or had to go in for a home delivery: (2) Post natal and new born visits – as per protocol; and (3) To check out on disease incidences reported to health worker or she he comes across during house visits especially where it is a notifiable disease.

Manpower
In order to provide above mentioned services, different categories of sub-centres should have the following personnel

Type of sub-centre	Sub-centre A		Sub-centre B (MCH Sub-centre)	
Staff	Essential	Desirable	Essential	Desirable
ANM/Health Worker (Female)	1	+1	2	
Health Worker (male)	1		1	
Staff Nurse(or ANM, if Staff Nurse is not available)				1**
Safari karamchari	1(part time)		1(full time)	

*To be outsourced

*If number of deliveries at the Sub-centre is 2 or more in a month.

3. Primary health centre level

The concept of primary health centre is not new to India. The Bhore committee in 1946 gave the concept of a primary health centre as a basic health unit, to provide, as close to the people as possible, an integrated curative and preventive health care to the rural population with emphasis on preventive and promotive aspects of health care.

The health planners in India have visualized the primary health centre and its sub-centres as the proper infrastructure to provide health services to the rural population. The Central Council of Health at its first meeting held in January 1953 had recommended the establishment of primary health centres in community development blocks to provide comprehensive health care to the rural population. The number of primary health centres established since then increased from 725 during the First Five Year Plan to 5484by the end of the Fifth Plan (1975-1980) each PHC covering a population of 100,000 or more spread over some 100 villages in each community development block. These centres where functioning as peripheral health service institution with little or no community involvement. Increasingly these centres came under criticism as they were not able to provide adequate health coverage, partly because they were poorly staffed and equipped and partly because they had to cover a large population of one lakh or more. The Mudaliar Committee in 1962 had recommended that the existing primary health centres should be scaled down to 40,000.

The declaration of Alma-Ata Conference in 1978 setting the goal of Health for All by 2000 AD has ushered in a new philosophy of equity, and a new approach, the primary health care approach. The National Health Plan (1983) proposed reorganization of primary health centres on the basis of one PHC for every 30,000 rural population in the plains, and one PHC for every 20,000 population in hilly, trial and backward areas for more effective coverage. As on March 2015, 25,308 primary health centres have been established in the country.

Functions of the PHC

The functions of the primary health centre in India cover the 8 'essential" elements of primary health care as outlined in the Alma-Ata Declaration. They are

1. Medical care;
2. MCH including family planning:
3. Safe water supply and basic sanitation
4. Prevention and control of locally endemic diseases.
5. Collection and reporting of vital statistics
6. Education about health
7. National Health Programmes as relevant:
8. Referral services:
9. Training of health, guides, health workers, local dais and health assistants; and
10. Basic laboratory services

Indian Public Health Standards for PHCs

The IPHS for primary Health Centres has been revised in 2012, keeping in view the resources available with respect to functional requirement for PHCs with minimum standards such as building, manpower, instruments and equipment drugs and other facilities etc. The standards prescribed are for a PHC covering 20,000-30,000 population with six beds, as all the block level PHCs are ultimately going to be upgraded as CHC with 30 beds of providing specialized services

The objectives of IPHS for PHCs are

i. To provide comprehensive primary health care to the community through the Primary Health Centres
ii. To achieve and maintain an acceptable standard of quality of care.
iii. To make the services more responsive and sensitive to the needs of the community

From service delivery angle, PHCs may be of two types, depending upon the delivery case load

Type A PHC: PHC with less than 20 deliveries per month.

Type B PHC: PHC with 20 or more deliveries per month.

All services have been classified as essential (minimum assured services) or desirable (which all states/UTs should aspire to achieve at this level of facility)

1. **Medical Care:**
   a. OPD services: 4 hours in the morning and 2 hours in the afternoon/evening. Time schedule will vary from state to state. Minimum OPD attendance should be 40 patients per doctor per day;
   b. 24 hours emergency services appropriate management of injuries and accident, First-aid, stabilization of the condition of patient before referral, dog bite/snake bite/scorpion bite cases, and other emergency conditions:
   c. Referral services; and
   d. In-patient services (6 beds)
2. **Maternal and child health care :**

ANTENATAL CARE :

(a) Early registration of pregnancy and minimum 4 antenatal check-ups according to suggested schedule. Ensure atleast one ANC, preferably the 3rd visit must be seen by a doctor,
(b) Minimum laboratory investigations such as haemoglobin, blood grouping and Rh typing, urine albumin and sugar and RPR test for syphilis
(c) Nutrition and health counselling:
(d) Supplementation of folic acid and iron tablets and tetanus toxoid immunization; brief advice on tobacco cessation, if antenatal mother is smoker or uses tobacco;
(e) Tracking of missed and left-out ANC:
(f) Identification of high risk pregnancies and appropriate management;
(g) Referral to First Referral Unit or other hospital in case of high risk pregnancy beyond the management capability of medical officer in PHC

INTRANATAL CARE

- ♥ 24 hours services for normal delivery;
- ♥ Promotion of institutional delivery:
- ♥ Conducting assisted deliveries including forceps and vacuum delivery whenever required:
- ♥ Manual removal of placenta; and
- ♥ Appropriate and prompt referral for cases needing specialist care;
- ♥ Management of pregnancy induced hypertension including referral:
- ♥ Pre-referred management in obstetric emergencies;
- ♥ Minimum 48 hours of stay after delivery:
- ♥ Managing labour using Partograph; and
- ♥ Proficient in identification of PPH eclampsia sepsis and prompt referral.

POSTNATAL CARE :

(1) Ensure post-natal care for 0 & 3rd day at the health facility both for the mother and new-born and sending direction to the ANM of the concerned area for ensuring 7th and 42nd day post-natal home visits 3 additional visits for a low birth weight baby (less than 2500 gm) on 14th day, 21st day and on 28th day:

(2) Initiation of early breast feeding within one hour of birth:

(3) Counselling on nutrition, hygiene, contraception essential new born care (As per guidelines of GOI on essential new-born care) and immunization;

(4) Others: Provision of facilities under Janani Suraksha Yojana (JSY); and

(5) Tracking of missed and left out PNC.

NEW BORN CARE:

(1) Facilities for Essential New Born Care (ENBC) and resuscitation (Newborn Care Corner in labour room /OT

(2) Early initiation of breast feeding within one hour of birth; and

(3) Management of neonatal hypothermia (provision of warmth/ Kangaroo Mother Care (KMC), infection protection, cord care and identification of sick newborn and prompt referral

CARE OF THE CHILD :

(a) Emergency care of sick child including Integrated Management of Neonatal and Childhood Illness (IMNCI);

(b) Care of routine childhood illness:

(c) Promotion of breast-feeding for 6 months;

(d) Full immunization of all infants and children against vaccine preventable diseases as per guidelines; and

(e) Vitamin A prophylaxis,

(f) Assess the growth and development of the infant and under 5 year children and make timely referral; and

(g) Management of severe acute malnutrition cases.

3. **Full range of** family planning services including counselling and appropriate referral for couples having infertility

4. **Medical termination of pregnancy** using manual vacuum aspiration technique, wherever trained personnel and facility exists

5**. Health education** for prevention and management of RTI/STI

6**. Nutrition Services**: Diagnosis and management of malnutrition, anaemia and vitamin A deficiency and coordination with ICDS.

7. School health services.

Essential : (1) screening of general health assessment of anaemia/nutritional status, visual acuity, hearing problem, dental check-up, physical disabilities, learning disorders and behaviour problems, etc: (2) Basic medicines to take care of common ailments, (3) Immunization as per national schedule: (4) Micronutrient (Vitamin A, iron and folic acid) management; deworming; and mid-day meal

Desirable: Health promoting schools.

8. **Adolescent health care**

To be provided preferably through adolescent friendly clinic for 2 hours once a week on a fixed day. Services should be comprehensive ie, a judicious mix of promotive, preventive, curative and referral services

Core package (essential): (a) Adolescent and reproductive health information, counselling and services related to sexual concerns, pregnancy, contraception abortion, menstrual problems etc.; (b) Services for tetanus immunization of adolescents; (c) Nutritional counselling prevention and management of nutritional anaemia; (d) STI l RTI management; and (e) Referral services for VCTC and PPTCT services and services for safe termination of p pregnancy. if not available at PHC

Outreach services in schools (essential) and community camps (desirable): Periodic health check-ups and health education activities, awareness generation and co-curricular activities
9. **Prevention and control of locally endemic diseases** like malaria, kala-azar, Japanese encephalitis etc (essential)

10.Collection and reporting of vital events (essential).

11. **Health education** and behavioural change communication

12. **Promotion of sanitation** including use of toilet and appropriate garbage disposal.

13. **Testing of water** quality and disinfection of water sources

14. **National health programmes.**

Revised National Tuberculosis Control Programme (RNTCP):

All PHCs to function as DOTS Centres to deliver treatment as per RNTCP treatment guidelines through DOTS providers and treatment of common complications of TB and side effects of drugs, record and report on RNTCP activities as per guidelines.

National Vector Borne Disease Control Programme

(a) Diagnosis of malaria cases, microscopic confirmation and treatment,

(b) Cases of suspected JE and dengue t provided symptomatic treatment, hospitalization and management as per the protocols. (c) Complete Kala-azar cases in endemic areas as per national

(d) Complete treatment of microfilaria positive cases DEC + albendazole and participation and arrangement of Mass Drug Administration (MDA) along with management of side reactions, if any Morbidity management of lymphedema cases

National AIDS Control Programme :

(a) IEC activities to enhance awareness and preventive measures about and HIV/AIDS, Prevention of Parents to Child Transmission (PPTCT) services. (b) Organizing school health educaton programme. (c) Screening of persons practicing high behaviour with one rapid test to be conducted at the PHC level and development of referral linkages with the nearest VCTC at the district hospital level for confirmation of HIV status of those found positive at one test stage in the high prevalence states. (d) risk screening of antenatal mothers with one rapid test for HIV and to establish referral linkages with CHC or district hospital for PPTCT services in the six high HIV prevalence states of Tamil Nadu, Andhra Pradesh, Maharashtra, Karnataka, Manipur and Nagaland. (e) Linkage with microscopy centre for HIV-TB 0coordination (f) Condom promotion and distribution of condoms to the high risk groups. (g) Help and guide patients with HIV/AIDS receiving ART with focus on adherence. (h) Pre and post-test counselling of AIDS patients by PHC staff in high prevalence states.

National Programme for Control of Blindness

(a) Basic services : Diagnosis and treatment of common eye diseases (b) Refraction services; and (c) Detection of cataract cases and referral for cataract surgery

National Leprosy Eradication Programme

Essential: (1) Health education to community regarding leprosy: (2) Diagnosis and management of leprosy and its complications including reactions; (3) Training of leprosy patients having ulcers for self-care; and (4) Counselling for leprosy patients for regularity/completion of treatment and prevention of disability

National Programme for Prevention and Control of Deafness (NPPCD)

Essential (1) Early detection of cases of hearing impairment and deafness and referral; (2) Basic diagnosis and treatment services for common ear diseases like wax in ear, otomycosis, otitis externa, ear discharge etc: and (3) IEC services for prevention, early detection of hearing impairment/deafness

National Mental Health Programme (NMHP)

Essential (a) Early identification (diagnosis) and treatment of mental illness in the community: (b) Basic services: Diagnosis and treatment of common mental disorders such as psychosis, depression, anxiety disorders and epilepsy, and referral; and (c) IEC activities for prevention, stigma removal, early detection of mental disorders and greater participation/role of community for primary prevention of mental disorders.

National Programme for Prevention and Control of Cancer Diabetes, CVD and Stroke (NPCDCS)

Cancer

Essential

- IEC services for prevention of cancer and early symptoms
- Early detection of cancer with warning signals like change in bladder/bowel habits, bleeding per rectum, blood in urine, lymph node enlargement, or thickness in breast, itching and/or redness or soreness of the nipple of breast, non-healing chronic sore or ulcer in oral cavity, difficulty in swallowing, obvious change in wart or mole, nagging cough or hoarseness of voice etc.
- Referral of suspected cancer cases with early warning signals for confirmation of the diagnosis

Desirable: PAP smear.

Other NCD Diseases

Essential:

- Health Promotion Services to modify individual group and community behaviour especially through (i)Promotion of healthy dietary habits; (ii) Increase physical activity; (iii) Avoidance of tobacco and alcohol; and (iv) Stress management
- Early detection, management and referral of diabetes mellitus, hypertension and other cardiovascular diseases and stroke through simple measures like history measuring blood pressure, checking for blood, urine sugar and ECG

Desirable:

Survey of population to identify vulnerable, high risk, and those suffering from disease.

National Iodine Deficiency Disorders Control Programme

Essential: (a) IEC activities to promote the consumption of iodated salt by the people; and (b) Monitoring of iodated salt through salt testing kits.

National Programme for Prevention and Control of Fluorosis (NPPCF)

Essential: (a) Referral services; and (b) IEC activities to prevent fluorosis

Desirable: (a) Clinical examination and preliminary diagnostic parameters assessment for cases of fluorosis if facilities are available and (b) Monitoring of village/community level activity

National Tobacco Control Programme (NTCP)

Essential: (a) Health education and IEC activities regarding harmful effects of tobacco use and second hand smoke (b) Promoting quitting of tobacco in the community and (c) making PHC tobacco free

Desirable: Watch for implementation of ban on smoking in public places, sale of tobacco products to minors, sale of products within 100 meters of educational institutions. IEC

National Programme for Health Care of Elderly

IEC activities on healthy aging (essential); and weekly geriatric clinic at PHC for providing complete health assessment of elderly persons, medicines, management of chronic diseases and referral services (desirable)

Oral health

health promotion, check-ups and appropriate referral on identification (essential)

15. **Appropriate and prompt referral** of cases needing special care and providing transport facilities either by PHC vehicle or other available referral transport. The funds should be made available for referral transport as per the provision under NRHM RCH-2 programme, and drop back home facility for patients under JSSK is mandatory

16**. Record of vital events**, reporting of births and deaths, and maintenance of all relevant records concerning services provided in PHC

17. **Training:** (a) Health workers and traditional birth attendants; (b) Initial and periodic training of paramedics in treatment of minor ailments; (c) Training of ASHAs; (d) Periodic training of doctors through continuing medical education, conferences, skill development training, etc. on emergency obstetric care: (e) Training of ANM and LHV in antenatal care and skilled birth attendance; (f) Training under Integrated Management of Neonatal and Childhood Illness (IMNCI); (g) Training of pharmacist on AYUSH component with standard modules; and (h) Training of AYUSH doctor in imparting health services related to National Health and Family Welfare programme

18. **Basic laboratory services** : (a) Routine urine, stool and blood tests (haemoglobin, CBC, blood grouping. Rh typing, blood sugar, blood cholesterol etc. (b) Bleeding time, clotting time; (c) Diagnosis of RTI STDs with wet mounting, Grams stain, etc.; (d) Sputum (c.) testing for tuberculosis (if designated as a microscopy centre under RNTCP); (e) Blood smear examination for malarial parasite; (f) Rapid tests for pregnancy; (g) RPR test for Syphilis surveillance; (h) Rapid diagnostic test for typhoid (Tyhpi Dot) and malaria: (i) Rapid test kit for faecal contamination of water; and () Estimation of chlorine level of water using ortho-toludine reagent

19. **Monitoring and supervision :** (a) Monitoring and supervision of activities of sub-centres through regular meetings/periodic visits, etc.: (b) Monitoring of all National Health Programmes: (c) Monitoring activities of ASHAs; (d) Medical officer should visit all sub-centres at least once in a month; and (e) Health assistants male and LHV should visit sub-centres once a week

20**. Selected surgical procedures**

The vasectomy, tubectomy (including laparoscopic tubectomy), MTP, hydrocelectomy and cataract surgeries as a camp/fixed day approach have to be carried out in a PHC having facilities of O.T.

21. Mainstreaming of AYUSH

22. Physical Medicine and Rehabilitation (PMR) services.

Desirable (a) Primary prevention of disabilities: (b) Screening, early identification and detection: (c) Counselling: and (d) Issue of disability certificate for obvious disabilities by PHC doctor

23. Maternal Death Review (MDR).

Desirable: Facility based MDR shall be conducted at the PHC

24. Functional Linkages with sub-centres

Essential

- There shall be a monthly review meeting at PHC chaired by MO (or in-charge), and attended by all the Health Workers (Male and Female) and Health Assistants (Male and female).
- On the spot Supervisory visits to sub-centres
- Organizing village health and nutrition day at anganwadi centres.

Desirable

- ASHAs and anganwadi workers should attend monthly review meetings. Medical officer should orient ASHAs on selected topics of health care

25. Monitoring and Supervision

Essential

- Monitoring and supervision of activities of sub-centre through regular meetings periodic visits, by LHV Health Assistant Male and Medical Officer etc.
- Monitoring of all National Health Programmes by medical officer with support of LHV Health Assistant Male and Health educator
- Monitoring activities of ASHAs by LHV and ANM (in her sub-centre area).

STAFFING PATTERN

The manpower that should be available at the PHC is as follows

Staff	Essential	
	Type A	Type B
Medical Officer MBBS	1	1
Medical Officer AYUSH		

Accountant cum data entry operator	1	1
Pharmacist	1	1
Pharmacist AYUSH		
Nurse-midwife (Staff-nurse)	3	4
Health worker (Female)	1*	1*
Health assistant (Male)	1	1
Health assistant (Female)/ lady health visitor	1	1
Health educator		
laboratory technician	1	1
cold chain & vaccine logistic assistant		
multi-skilled group D worker	2	2
sanitary worker cum watchman	1	1
total	13	14

*For sub-centre area of PHC

HEALTH PROMOTION

Definition

Health promotion is the process of enabling people to increase control over, and to improve, their health. It moves beyond a focus on individual behaviour towards a wide range of social and environmental interventions. Health promotion is the process of enabling people to increase control over, and to improve, their health. It moves beyond a focus on individual behaviour towards a wide range of social and environmental interventions (WHO)

First international conference on health promotion was held in Ottawa in November 1986, primarily in response to growing expectation for a new public health movement around the world. It was built on progress made through Declaration on primary health care at Alma-Ata, and the debate at the World Health Assembly on intersectoral action for health. The conference

resulted in proclamation of the Ottawa Charter for Health Promotion, which has been a source of guidance and inspiration for health promotion since that time.

Health is a basic human right and is essential for social and economic development. Increasingly health promotion is being recognized as an essential element of health development. Health promotion, through investment and action, has a marked impact on the determinants of health so as to create the greatest health gain for people, to contribute significantly to the reduction of inequities of health, and to further human rights. The ultimate goal is to increase health expectancy.

The Jakarta Declaration on Health Promotion (the fourth conference held in July 1997) offered a vision and focus for health promotion into the 21st century. The determinants of health; new challenges in the 21st century; and the fundamental conditions and resources for health are peace, shelter, education, social security, social relations, food, income, the empowerment of women, a stable ecosystem, sustainable resource use, social justice, respect for human rights, and equity. Above all, poverty is the greatest threat to health.

Demographic trends such as urbanization, an increase in the number of older people and the high prevalence of chronic diseases pose new problems in all countries. Other social, behavioural and biological changes such as increased sedentary behaviour, resistance to antibiotics and other commonly available drugs, increase drug abuse, and civil and domestic violence threaten the health and well-being of hundreds of millions of people. New and re-emerging infectious diseases, and the greater recognition of mental health problems, require an urgent response. It is vital that approaches to health promotion evolve to meet changes in the determinants of health. To address emerging threats to health, new forms of actions are needed. The challenges for the coming years will be to unlock the potential for health promotion inherent in many sectors of society, among local communities, and within families.

The Ottawa Charter incorporate five key action areas in health promotion. They are:

1. Build healthy public policy
2. Create supportive environment for health,
3. Strengthen community action for health,
4. Develop personal skills, and
5. Re-orient health services

Build healthy public policy

Health promotion goes beyond health care. It puts health on the agenda of policy makers in all sectors and at all levels, directing them to be aware of the health consequences of their decisions and to accept their responsibilities for health.

Create supportive environment for health

Systematic assessment of the health impact of a rapidly changing environment- particularly in areas of technology, work, energy production and urbanization- is essential and must be followed by action to ensure positive benefit to the health of the public. The protection of the natural and built environments and the conservation of natural resources must be addressed in any health promotion strategy.

Strengthen community action for health

Health promotion works through concrete and effective community action in setting priorities, making decision, planning strategies and implementing them to achieve better health. At the heart of this process is the empowerment of communities – their ownership and control of their own endeavours and destinies.

Develop personal skills

Health promotion supports personal and social development through providing information, education for health, and enhancing life skills. By so doing, it increases the options available to people to exercise more control over their own health and over their environment, and to make choices conductive to health.

Reorient health services

The responsibility for health promotion in health services is shared among individuals, community groups, health professionals, health service institutions and governments. They must work together towards a health care system which contributes to the pursuits of health. The role of the health sector must move increasingly in a health promotion direction, beyond its responsibility for providing clinical and curative services. Health services need to embrace an expanded mandate which is sensitive and respects cultural needs. This mandate should support the needs of individuals and communities for a healthier life, and open channels between the health sector and broader social, political, economic and physical environmental components.

It also incorporates three basic strategies for health promotion, "enabling, mediating and advocacy", which are needed and applied to all health promotion action areas. They are

Advocate

Good health is a major resource for social, economic and personal development, and an important dimension of quality of life. Political economic, social, cultural, environmental, behavioural and biological factors can all favour health, or be harmful to it. Health promotion action aims at making these conditions favourable through advocacy for health.

Enable

Health promotion focusses on achieving equity on health. Health promotion action aims at reducing differences in current health status and ensuring equal opportunities and resources to enable all people to achieve their fullest health potential. This includes a secure foundation in a supportive environment, access to information, life skills and opportunities for making healthy choices. People cannot achieve their fullest potential unless they are able to take control of those things which determine their health. This must apply equality to women and men.

Mediate

The prerequisites and prospects for health cannot be ensured by the health sector alone. More importantly, health promotion demands coordinated action by all concerned: by governments, by health and other social and economic sectors, by non-governmental and voluntary organizations, by local authorities, by industry and by the media. People in all walks of life are involved as social groups and health personnel have a major responsibility to mediate between differing interests in society for the pursuit of health.

A Logo was created for Ottawa Conference. Since then, WHO kept this symbol as the Health Promotion Logo, as it stands for the approaches to health promotion as outlined in Ottawa Charter. The Logo represents a circle with 3 wings. It incorporates five key action areas in health promotion and three basic health promotion strategies.

Health promotion strategies and programmes should be adapted to the local needs and possibilities of individual countries and region to take into account differing social, cultural and economic systems.

MILLENNIUM DEVELOPMENT GOALS (MDGs)

In the Millennium Declaration of September 2000, Member States of the United Nations made a most passionate commitment to address the crippling poverty and multiplying misery that grip many areas of the worlds. Governments had set a date of 2015 by which they would meet the millennium achieve universal primary education, promote gender equality and empower women, reduce child mortality, improve maternal health, combat HIV/AIDS, malaria and other diseases, ensure environmental sustainability and develop a global partnership for development.

SUSTAINABLE DEVELOPMENT GOALS

The 2030 Sustainable Development Agenda is of unprecedented scope and ambition, applicable to all countries, and goes well beyond the MDGs. While poverty eradication, health, education, and food security and nutrition remain priorities, the Sustainable Development Goals (SDGs) comprise a broad range of economic, social and environmental objectives, and offer the prospect of more peaceful and inclusive societies.

Paragraph 26 of the 2030 agenda for sustainable development addresses health as follows:

To promote physical and mental health and well-being, and to extend life expectancy for all, we must achieve universal health coverage and access to quality health care. No one must be left behind. We commit to accelerating the progress made to date in reducing new born, child and maternal mortality by ending all such preventable deaths before 2030. We are committed to ensuring universal access to sexual and reproductive health care services, including for family planning, information and education. We will equally accelerate the pace of progress made in fighting malaria, HIV/AIDS, tuberculosis, hepatitis, Ebola and other communicable diseases and epidemics, including by addressing growing anti-microbial resistance and the problem of unattended diseases affecting developing countries. We are committed to the prevention and treatment of non-communicable diseases, including behavioural, developmental and neurological disorders, which constitute a major challenge for sustainable development.

Out of the 17 goals 3rd goal is devoted specifically to health, and is framed in deliberately broad terms that are relevant to all countries and all populations. “Ensure healthy lives and promote well-being for all at all ages”.

HEALTH POLICY

Policies are general statements based on human aspiration, set of values, commitments, assessment of current situations and an image of a desired future situation. A national health policy is an expression of goals for improving the health situation, the priorities among these

goals, and the main directions for attaining them, Health policy is often defined as the national level.

Each country will have to develop a health policy of its own aimed at defined foals, for improving the people's health, in the light of its own problems, particular circumstances, social and economic structures, and political and administrative mechanisms. Among the crucial factors affecting realization of these goals are: a political commitment; financial implications; administrative reforms; community participation and basic legislation.

A landmark in the development of health policy was the worldwide adoption of the goal of HFA by 2000 A.D. a further landmark was the Alma-Ata Declaration (1978) calling on all governments to develop and implement primary health care strategies to attain the target of "HFA" by 2000 A.D. and more recently, Millennium Development Goals.

HEALTH SERVICE RESEARCH

Health research has several ramifications. It may include

(a) Biomedical research, to elucidate outstanding health problems and develop new or better ways of dealing with them; (b) Intersectoral research, for which relationships would have to be established with the institutions concerned with the other sectors, and (c) Health services research or health practice research (now called "health systems research").

The concept of health services research (HSR) was developed during 1981-1982. It has been defined as "the systematic study of the means by which biomedical and other relevant knowledge is brought to bear on the health of individuals and communities under a given set of conditions". HSR is wide in scope. It deals with all aspects of management of health services, viz. prioritization of health problems, planning, management, logistics and delivery of health care services. It deals with such topics as manpower, organization, the utilization of facilities, the quality of health care, cost-benefit and cost-effectiveness.

Thousands of people suffer morbidity, mortality and disability not because of deficiencies in biomedical knowledge but as a result of the failure to apply this knowledge effectively. Health services research aims to correct this failure.

The concept of HSR is holistic and multidisciplinary. The prime purpose of HSR is to improve the health of the people through improvement not only of conventional health services but also of other services that have a bearing on health. HSR is essential for the continuous evolution and refinement of health services

CONCLUSION

Health care is an expression of concern for fellow human beings. It is defined as a "multitude of services rendered to individuals, families or communities by the agent of the health service or professions, for the purpose of promoting, maintaining, monitoring or restoring health". Such services might be staffed, organized, administered and financed in every imaginable way, but they all have one thing in common: people are being "served", that is, diagnosed, helped, cured, educated and rehabilitated by health personnel. In many countries, health care is completely or largely a government function.

Health care includes “medical care”. Many people mistakenly believe that both are synonymous. Medical care is a subset of a health care system. The term “medical care (which ranges from domiciliary care to resident hospital care) refers chiefly to those personnel services that are provided directly by physician or rendered as a result of the physician’s instructions”.